

EVERGREEN HEALTH SYSTEM

420 Green Valley Rd, Portland, OR 97219 | (503)555-6605 | www.evergreenhs.org

DISCHARGE SUMMARY

Patient Name: Folake Adesanya	DOB: October 15, 1942 (Age 83)
MRN: MRN-2140124	Gender: Male
Admission Date: August 20, 2025	Discharge Date: September 08, 2025 (19 days)
Attending Physician: Dr. Valentina Russo, MD — Transplant Medicine	Insurance: Aetna HMO #AET-E5-55555
ICU Admission: YES — MICU/SICU	ICU Days: 13 of 19 total days

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Acute Liver Failure — Drug-induced (DILI)	K71.10	Acute / Critical
HIV/AIDS — CD4 < 50 cells/μL	B24	Chronic / Complicating
Active Alcohol Use Disorder	F10.20	Chronic / Complicating

HOSPITAL COURSE

Folake, a 83-year-old male with a history of HIV/AIDS — CD4 < 50 cells/μL and Active Alcohol Use Disorder, was admitted to the Neuro-ICU following witnessed generalised tonic-clonic seizure progressing to status epilepticus refractory to first and second-line benzodiazepines. The patient required propofol infusion for burst-suppression. EEG confirmed continuous electrographic seizures for > 30 minutes. Levetiracetam IV 3000 mg/day and lacosamide IV 400 mg/day were initiated. MRI brain identified a new cortical lesion requiring biopsy. The patient achieved seizure control by day 3 and was extubated on day 4. Neurosurgery and neuro-oncology consultations were obtained. Discharged on optimised anti-epileptic regimen with outpatient neuro-oncology follow-up.

VITALS AT DISCHARGE

■ NOTE: Several vital signs remain outside normal limits at time of discharge — close monitoring required.

BP	HR	SpO2	Temp	RR	Weight
109/61 mmHg	100 bpm	91% (on 4L O2)	99.6°F	23/min	114 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Levetiracetam	1500 mg	Twice daily	Ongoing — neurology follow-up
Propofol	Per protocol	Continuous IV sedation	Discontinued prior to discharge
Rivaroxaban	20 mg	Once daily with evening meal	Indefinite — PE management
Lacosamide	200 mg	Twice daily	Ongoing — neurology follow-up
Metronidazole	500 mg	Every 8 hours	7-day course
G-CSF (Filgrastim)	480 mcg SC	Once daily	Until ANC > 1.0 x10 ³ /μL

FOLLOW-UP PLAN

URGENT: Transplant Medicine: Dr. Valentina Russo, MD — within 72 hours of discharge (September 11, 2025).

Primary Care: Within 1 week (September 15, 2025). Full labs including BMP, CBC, LFTs.
Home health nursing: Daily vitals and wound check for first 7 days post-discharge.
Inpatient rehabilitation / LTACH: Transfer arranged — starts September 22, 2025.

■ **HIGH-RISK DISCHARGE — Patient requires close monitoring. Re-admission risk: HIGH.**

Attending Physician Signature: _____

Dr. Valentina Russo, MD — Transplant Medicine | License #: OR-MD-65789 | Date: September 08, 2025

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